



Standard Operating Procedure: Website Inquiry Management & Physician Escalation

Version: 1.0

Effective Date: April 10, 2026

Review Date: March 17, 2027

Scope: Clinical Operations & Administrative Staff

Purpose

To establish a streamlined, professional workflow for all digital inquiries and define a clear hierarchy for physician communications. This ensures that routine inquiries are handled with agility while high-priority clinical or operational issues are funneled to the appropriate leadership immediately.

Inquiry Classification & Response Times

All website inquiries are consolidated into the primary inquiry email. Personnel must triage and respond based on the following benchmarks:

- **Inbound Patient Leads:** General requests or patient inquiries.
Target Response: Within **2–4 hours**.
- **Physician Partnerships:** Doctors seeking to affiliate for patient referrals.
Target Response: Within **2 hours** (High Priority).



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INTERNAL POLICY: THE 4-HOUR WORKFLOW OPTIMIZER

Effective Date: March 16, 2026

Purpose

The purpose of this protocol is to maintain high operational momentum and maximize patient conversion rates by implementing a **"Real-Time Dispatch"** strategy. This SOP establishes the **4-Hour Rule** as a mandatory buffer to prevent **lead fatigue**, ensure the High-Priority Action Queue remains fluid, and eliminate the accumulation of administrative **"pile-ups."** By prioritizing immediate outreach, we ensure patients are reached while their intent is highest, reducing "phone tag" and increasing return on investment (ROI).

The Core Principle: "Don't Batch, Dispatch"

Staff are strictly prohibited from holding leads until the end of a shift.

Real-Time Processing: Every lead entering the High-Priority Action Queue must be addressed as it arrives.

The Buffer Rule: While the "4-hour" window is the maximum allowable buffer, the performance standard is immediate action.

The High-Priority Action Sequence

Once a lead is identified, the following 3-step sequence must be completed within the 4-hour window:

Step 1: The Initial Call (Priority One)

- **Action:** Initiate a phone call immediately upon receipt of the lead.
- **Objective:** Secure a 5-minute conversion while the patient is likely still near their device.



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Step 2: Documentation & Voicemail

- **Voicemail:** If the patient does not answer, leave a professional, concise voicemail identifying the organization and the reason for the call.
- **Trello Update:** Add a timestamped comment in the card detailing the call attempt and the outcome (e.g., "UPDATE AS OF 3/16/26 @ 10:00 AM: Call 1 - No answer, VM left. -").

Step 3: The Email Sequence

- **Action:** Immediately following the voicemail and Trello update, trigger the corresponding email template.
- **Context:** The email should reference the missed call and provide a clear "Call to Action" (CTA) to facilitate a self-service conversion.

Preventing "Lead Fatigue" & Pile- ups

To maintain a "Clear Desk" policy, employees must adhere to the following:

- **Emergency Prevention:** Treat every new lead as a "perceived emergency" to prevent a simple task from escalating into a backlog.
- **Status Audits:** The Clinical Operations Lead will review the Trello Board every 4 hours to ensure no lead has remained in the "High-Priority Action Queue" beyond the buffer limit.

ROI & Conversion Metrics

The success of this SOP is measured by:

Speed to Lead: Time elapsed between lead entry and first call.

One-Call Resolution: Ratio of immediate conversions vs. multi-day "phone tag" cycles.



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The Chain of Command

Tier 1: The First Point of Contact (Coordinator)

- **Primary Responsibility:** Inbox Management & Routine Outreach.
- **Daily Action:** Monitor the physician outreach inbox.
- **Scope:** Answer general questions, send out standard program info, and handle initial "First-Touch" greetings.
- **When to Escalate:** If a physician asks a question regarding specific logistics, scheduling conflicts, or patient support issues that you cannot answer using the standard FAQ.

Tier 2: Logistics & Patient Support Lead (Ms. Carina)

Primary Responsibility: Operational & Logistics Resolution.

- **Scope:** Acts as the "Senior Escalation Authority." She handles complex operational inquiries, difficult scheduling situations, or specific patient support hurdles that exceed the coordinator's scope.
- **Action:** Resolves the issue or provides the coordinator with the specific instructions needed to reply.
- **When to Escalate:** If the issue is a **critical clinical error**, a major medical safety concern, or an urgent matter that Ms. Carina cannot resolve within her operational authority.



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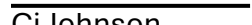
Tier 3: Clinical Operations Lead (Rheinalei)

- **Primary Responsibility:** Final Authority & Urgent Clinical Resolution.
- **Scope:** Handles all matters requiring **urgent attention** and high-level clinical oversight.
- **Action:** Takes over the concern to resolve it as quickly as possible.
- **Requirement:** This stage is only reached after Tier 1 and Tier 2 have attempted resolution, or if the matter is so urgent it threatens patient safety or physician partnerships.

4. Escalation Flowchart Level	Person	Role in Escalation
Level 1	Coordinator	Triage inbox & handle routine emails.
Level 2	Ms. Carina	Solve complex logistics & operational issues.
Level 3	Rheinalei	Resolve urgent clinical/high-priority concerns.

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